

Competency Questions Using GPT 5.5 Medium

CQ1. Can FASTO represent CGM-derived glycemic metrics for a patient, including time in range, time below range, time above range, glucose management indicator, coefficient of variation, mean glucose, and the percentage of active CGM wear time?

CQ2. Can FASTO classify hypoglycemia events into Level 1, Level 2, and Level 3 hypoglycemia according to glucose thresholds and need for assistance?

CQ3. For each patient taking insulin or otherwise at high risk for hypoglycemia, can FASTO represent that glucagon is prescribed and that family, caregivers, school personnel, or other support persons know its location and administration procedure?

CQ4. Can FASTO represent the recommended initial treatment protocol for conscious hypoglycemia: oral glucose-containing carbohydrate, repeat treatment after 15 minutes if hypoglycemia persists, and avoidance of high-fat or high-protein foods for initial treatment?

CQ5. Can FASTO identify patients with type 1 diabetes for whom fear of hypoglycemia is a documented barrier to achieving glycemic targets or well-being?

CQ6. Can FASTO distinguish pediatric type 1 diabetes care plans from adult care plans so that pediatric plans can encode age-appropriate and developmentally appropriate care rather than inheriting adult management assumptions?

CQ7. Can FASTO represent that glycemic status assessment frequency should increase during rapid growth and development in children and adolescents, and for recent treatment changes or frequent/severe hypoglycemia or hyperglycemia?

CQ8. Can FASTO support annual screening of older adults with diabetes for geriatric syndromes, hypoglycemia, and polypharmacy, and relate those findings to individualized goals and therapeutic approaches?

CQ9. Can FASTO represent that most adults with type 1 diabetes should be treated with continuous subcutaneous insulin infusion or multiple daily doses of basal plus prandial insulin, including injected or inhaled prandial insulin?

CQ10. Can FASTO represent mealtime insulin dose adjustment based on carbohydrate intake, fat and protein intake, concurrent glycemia, glycemic trends, sick-day management, and anticipated physical activity?

CQ11. Can FASTO model automated insulin delivery systems as devices that integrate an insulin pump, CGM, and an algorithm to modulate insulin delivery, and represent caregiver skill/preference as part of device selection when the patient is a young child or otherwise dependent?

CQ12. Can FASTO represent adverse events and patient-reported outcomes associated with advanced hybrid closed-loop systems, such as severe hypoglycemia, ketoacidosis, skin problems from adhesives or infusion sets, and treatment satisfaction?

CQ13. Can FASTO represent low-dose glucagon as a preventive or treatment option for exercise-associated hypoglycemia in adolescents and adults with type 1 diabetes, including efficacy outcomes and adverse events?

CQ14. Can FASTO represent presymptomatic type 1 diabetes monitoring using A1C approximately every 6 months, annual 75-g oral glucose tolerance testing, optional CGM metrics, and risk stratification by age and number/type of islet autoantibodies?

CQ15. Can FASTO represent sick-day or stressful-event management for ketosis-prone individuals, including more frequent glucose monitoring, urine or blood ketone monitoring, treatment reevaluation, and DKA risk mitigation?

CQ16. Can FASTO represent shared decision-making in diabetes treatment decisions using patient values, preferences, prognosis, comorbidities, social determinants of health, treatment burden, and informed financial considerations?

CQ17. Can FASTO represent dietary-pattern interventions for type 1 diabetes, especially high-fiber and carbohydrate-restricted patterns, and link them to outcomes such as HbA1c, time in range, coefficient of variation, hypoglycemia, insulin dose, and anthropometric measures?

CQ18. Can FASTO represent the transition from pediatric to adult diabetes care as a care phase with possible gaps in ambulatory care and elevated DKA risk for adolescents and young adults?